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Strengthening Long-Term Care Systems: WHO's Strategic Approach and the Development of Global Standards

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A B S T R A C T

Population aging represents one of the most significant social transformations of our time, with profound implications for health systems, social protection, and intergenerational solidarity. By 2050, 1 in 6 people globally will be aged 60 years or older, with the fastest growth occurring in low- and middle-income countries least prepared to respond. Alongside demographic shifts, sociocultural transitions—urbanization, changing family structures, and increased women's labor participation—are fundamentally altering traditional caregiving arrangements. The COVID-19 pandemic exposed with devastating clarity the consequences of neglected long-term care (LTC) systems. The United Nations has recognized LTC as a global public health priority, establishing it as 1 of 4 action areas under the UN Decade of Healthy Ageing (2021-2030). Building on a decade of normative work—including the LTC System Framework and the LTC Service Package—World Health Organization is now advancing the development of Global Standards for LTC, with a consultation draft released in May 2026 and currently open for public input. The proposed standards address interconnected domains spanning foundational principles; service delivery across home, community, and residential settings; health and care workforce; support for unpaid carers; governance; financing; and quality monitoring. This article describes the strategic approach to strengthening LTC systems globally, presents the evolving Global LTC Standards framework, and outlines pathways for clinicians, researchers, policymakers, and civil society to engage in shaping these standards. The implications extend across practice, policy, and research—offering opportunities to advance quality, equity, and sustainability in LTC worldwide.

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The Imperative for Long-Term Care

The world is aging at an unprecedented pace. The number of people aged 60 years and older is projected to reach 2.1 billion by 2050, with the proportion of those aged 80 and above growing fastest of all.[1] This demographic transition is not confined to high-income economies; by 2050, nearly 80% of older people will live in low- and middle-income countries, many of which lack the health and social infrastructure to respond.[2]

Concurrent sociocultural shifts are transforming the landscape of care. Urbanization and migration separate families across distances; declining fertility rates shrink the pool of potential family carers; and women's increased participation in the labor market reduces their availability for unpaid caregiving, a role they have historically shouldered disproportionately. These changes reflect broader social and economic transformations that, while advancing gender equality and economic participation, simultaneously erode traditional caregiving arrangements. They necessitate new, organized approaches to delivering and financing long-term care (LTC), supported by public policy and adequate investment.

LTC encompasses the activities undertaken to ensure that people who experience significant loss in intrinsic capacity can maintain a level of functional ability consistent with their basic rights, fundamental freedoms, and human dignity.[3,4] It includes personal, social, and medical services—from assistance with daily activities to skilled nursing and rehabilitation—provided across a continuum of settings from people's own homes to community centers and residential facilities, and delivered by a workforce spanning paid professionals to unpaid carers such as family members and neighbors. This diversity of needs, services, and settings underscores the importance of integration across health and social care sectors.[5]

The COVID-19 pandemic revealed, with tragic clarity, the costs of neglecting LTC systems. Residential care facilities became epicenters of transmission and death; in some countries, residents of LTC facilities accounted for more than half of all COVID-19 deaths.[6,7] Care

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workers—predominantly women, often migrants, frequently employed under precarious conditions—faced dangerous working environments with inadequate protection.[8,9] The pandemic did not create these vulnerabilities; it exposed them. And it demonstrated that LTC is not a peripheral concern but a matter of public health, human rights, and social solidarity.

Equity considerations compound these challenges. Access to quality LTC varies dramatically by income, geography, gender, and social status. Any credible response to the LTC imperative must place equity at its center.

The Foundation for Global Action

The global policy architecture for LTC has evolved substantially over the past decade. The 2015 World Report on Ageing and Health introduced the public health framework of healthy aging—centered on functional ability rather than the absence of disease—and positioned LTC system strengthening as an essential infrastructure for enabling older people to live the lives they value.[3,4] The 2016 Global Strategy and Action Plan on Ageing and Health, endorsed by the World Health Assembly, translated this vision into commitments that Member States continue to pursue.[10]

The UN Decade of Healthy Ageing (2021-2030), led by World Health Organization (WHO) in collaboration with other UN agencies, provides the current organizing framework.[11,12] The Decade identifies 4 interconnected action areas, the fourth of which—ensuring access to LTC for those who need it—directly underpins our current work. At its midpoint in 2026, Member States face a critical window to accelerate investment in LTC systems if they are to demonstrate meaningful progress by 2030.

Foundational technical tools have been developed to support countries in this effort. *Framework for countries to achieve an integrated continuum of long-term care* (2021) provides a conceptual architecture for understanding how LTC systems function, identifying key building blocks, including governance, financing, service delivery, workforce, innovation, and research.[5] *Long-term care for older people: package for universal health coverage* (2024) offers operational guidance on the range of services that should be available across the care continuum and diverse care settings.[13]

These products are complemented by country-level engagement: policy dialogs and technical assistance including LTC system assessments and policy reviews conducted in partnership with national governments, as well as collaboration with international partners, including the International Labor Organization on workforce conditions and the Organization for Economic Co-operation and Development on comparative system analysis.[14,15] The development of Global Standards for LTC represents the next step in this trajectory.

The Global LTC Standards Initiative

Rationale

Despite growing recognition of LTC as essential to health systems, substantial variability persists in how LTC is defined, organized, and quality assured across countries.[14,16] Many countries lack comprehensive regulatory frameworks for LTC, whereas others have developed standards in isolation without the benefit of cross-national evidence or learning. Important work on LTC quality and workforce has been advanced by other international organizations, including the International Labor Organization and the Organization for Economic Co-operation and Development. The Global LTC Standards initiative complements these efforts by establishing a shared reference point—not as prescriptive mandates but as an aspirational framework that countries can adapt to their contexts and capacities.

Scope and Structure

The proposed standards are organized around interconnected domains that address LTC holistically. Foundational definitions and principles establish the cross-cutting concepts—a rights-based approach, person-centeredness, autonomy, and an integrated continuum of care—that underpin the entire framework. Building on this foundation, domain-specific standards address the quality of services and environment in both home and community-based care settings and residential care facilities. A further set of domains focuses on those who provide care: the paid LTC workforce, encompassing workforce planning, competency-based training, decent working conditions, and career progression; and unpaid carers, addressing their identification, respite care, skills training, and social protection. System-level enablers—governance, financing, and quality monitoring—complete the framework, establishing the structural conditions necessary for sustainable and accountable LTC systems.

Development and Design

We are developing the standards through a multistage process: comprehensive evidence reviews synthesizing global literature; analysis of existing national standards and policies across WHO regions; iterative consultation with Member States, civil society organizations, and academic experts; and technical refinement through expert working groups. Countries across income levels have contributed to this process, sharing their national experience and helping to validate the standards' relevance to diverse health system contexts.

The framework is conceived as adaptable rather than uniform.[17] Standards establish principles and expected practices that apply universally, while allowing countries to determine the specific tools, processes, and service configurations suited to their capacity and context. This approach has resonated across settings: participants from countries spanning high-, middle-, and low-income economies have actively engaged in the development process.

Discussions are also ongoing within WHO on how the life-course approach can inform LTC across different age groups and populations with LTC needs.[18]

Consultation and Pathways for Engagement

The year 2026 marks the midpoint of the UN Decade of Healthy Ageing—a critical juncture for countries to intensify their investment in LTC system strengthening. The consultation draft of the Global LTC Standards was released on May 18, 2026, and a global public consultation is open through November 30, 2026.

The consultation is modeled on the structured public processes used by countries in developing their own national LTC standards—for instance, Canada's development of national standards engaged over 20,000 participants through surveys, focus groups, and targeted consultations. The global consultation operates through multiple channels: an online questionnaire (initially in English, with all 6 WHO official languages available during the consultation period), written submissions to ltc@who.int, and regional and partner-convened consultation events across WHO regions. The consultation draft is also being disseminated through WHO regional and country offices to reach national focal points and local stakeholders.

We particularly invite engagement from the readership of this journal. Clinicians and medical directors can contribute expertise on standards relating to clinical care quality, geriatric assessment, palliative care, and the clinical dimensions of infection prevention and control. Researchers can support evidence synthesis, identify

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gaps in the current knowledge base, and contribute to implementation science as countries begin adapting the standards. Professional associations can serve as conduits for member input while considering how global standards might inform specialty training, certification, and continuing education requirements (Table 1).

Following the consultation, in-country assessments and comparative analyses during 2026 to 2027 will test the standards' applicability across different health system contexts and resource levels, while generating evidence on LTC system readiness that can inform national policy development.[19] Finalization and publication of the Global LTC Standards is anticipated around the second half of 2027.

Looking Ahead: Emerging Priorities for LTC Systems

The Global LTC Standards address the current state of knowledge and practice, but several emerging priorities will shape the future of LTC systems and deserve sustained attention.

The rebalancing of care toward home and community-based models is one such priority. While residential facilities remain essential components, the preference of most older people—and the most sustainable path for most health systems—is to support aging in the right place through robust community services.[20] How to design, finance, and scale effective community-based care across diverse contexts remains one of the most consequential questions facing LTC policy.

Recognition and support for unpaid carers require urgent, sustained action. Globally, the majority of LTC is provided by family members—predominantly women—who often sacrifice employment, retirement savings, and their own health.[15,21] Despite their indispensable role, unpaid carers remain largely invisible in health systems planning. Addressing this gap is not only a matter of equity but also of system sustainability.

Pandemic preparedness in LTC settings has emerged as a defining concern of this decade. The disproportionate mortality among care facility residents and the outbreaks that spread through inadequately protected facilities demonstrated that LTC must be integrated into broader public health emergency preparedness.[6,7,22] The goal is not only to respond better to the next crisis but also to build a resilience that improves care quality during ordinary times.

Sustainable financing underpins all other objectives. Without adequate and equitable funding mechanisms—including social insurance, tax-based approaches, and integration with broader social protection systems—LTC systems will remain unable to deliver on their promise of quality, accessible care.[23,24]

Implications for Practice, Policy, and Research

Practice

The Global LTC Standards can offer practitioners a reference framework for quality improvement in both facility and community settings.[17,25] Medical directors and clinical leaders can benchmark current practices against standards for clinical care, infection prevention, and person-centered care. The emphasis on multidisciplinary care coordination invites reflection on how physicians, nurses, social workers, rehabilitation professionals, and family carers can function as integrated teams rather than parallel silos. The explicit inclusion of freedom from restraint, meaningful activity, and social connection as standards—not merely aspirations—challenges facilities to examine practices that may have become routine but fail to serve residents' well-being.

Policy

For policymakers, the Global LTC Standards provide an internationally validated framework for developing national standards. Countries developing or revising LTC legislation and regulations can draw on the standards' comprehensive scope while adapting specific elements to local contexts. The governance and financing domains offer particular value for policymakers grappling with regulatory design and sustainable funding mechanisms. Importantly, the standards are intended to support rather than supplant national policy processes—providing a common reference that facilitates cross-national learning while respecting each country's authority over its own approach.

Research

The standards initiative opens multiple research avenues. Implementation science studies can examine how standards are adapted, adopted, and translated into practice across different settings and health system contexts. Comparative effectiveness research can evaluate which interventions best achieve standards-related outcomes. Health economics research on the costs and benefits of implementation can inform resource allocation decisions. The standards also highlight evidence gaps—including in areas such as digital health technologies and artificial intelligence applications in LTC, outcomes measurement across diverse populations, and cost-effectiveness of alternative service delivery models—that merit priority attention from the research community.

Table 1
Pathways for Stakeholder Engagement in the WHO Global LTC Standards Consultation (2026)

Stakeholder Group	Relevant Standards Domains	Engagement Channels
Clinicians and medical directors	Clinical care, needs assessment, infection prevention, and palliative care	Online survey, written submissions, and professional association events
Nurses and allied health professionals	Care planning, workforce standards, and person-centered care	Online survey, regional events, and professional associations
Social workers	Home and community-based care, unpaid carer support, and rights-based approaches	Online survey, civil society consultation events
Facility administrators and providers	Residential care standards, governance, and quality monitoring	Written submissions, regional events, and provider networks
Researchers and academics	Evidence synthesis, implementation science, and outcomes measurement	Written submissions, WHO Collaborating Centres network
Policymakers and government officials	Governance, financing, and regulatory frameworks	Member State consultations, regional events
Older people and carers	All domains, particularly person-centeredness and unpaid carer support	Civil society events, online survey, and regional events

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Conclusions

The development of Global Standards for LTC represents a significant milestone in efforts to strengthen health and social care systems' response to population aging. Yet standards alone do not create change; they provide a shared reference point around which countries and stakeholders can mobilize. The standards are designed to complement, not replace, existing national frameworks and other international efforts—facilitating cross-national learning while respecting each country's authority over its own approach.

The impact of this initiative will depend on broad engagement: from Member States who will adapt standards to their national contexts, from practitioners and researchers whose expertise grounds the standards in reality, from civil society organizations that amplify the voices of those most affected, and from older people and carers themselves. The consultation draft of the Global LTC Standards is open for public input until November 30, 2026, and we invite all who share a commitment to quality LTC to participate in shaping a framework that can guide progress toward a world in which every person can age with dignity, security, and the support they need.

The consultation draft and information on how to participate are available on the WHO website[26] or by writing to ltc@who.int. Readers wishing to be notified directly are also welcome to contact the corresponding author.

Declaration of Generative AI and AI-Assisted Technologies in the Manuscript Preparation Process

During the preparation of this work, the authors used Claude (Anthropic) in order to assist with manuscript drafting, text refinement, and formatting. After using this tool, the authors reviewed and edited the content as needed and take full responsibility for the content of the published article.

Disclaimer

The authors are staff members of the WHO. The views expressed in this article are those of the authors and do not necessarily represent the decisions, policy, or views of the WHO.

Disclosure

The authors declare no conflicts of interest.

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